

Dynamic Guideline: WHO-Based Abortion Care - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Scope statement

This report synthesizes **exactly 5 WHO abortion-care-related normative documents/guidelines** identified in the supplied evidence summaries, with emphasis on the most recent WHO item requested: *Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)*, published by WHO on **11 December 2025** [WHO B09645](#).

Important interpretation: the 11 December 2025 WHO youth-engagement publication is an **evidence brief/case study about guideline development methodology**, not a clinical treatment guideline. The latest WHO **clinical** abortion-care guideline remains the *Abortion care guideline, 2nd ed.*, published on **24 August 2025**, with an executive summary published on **13 October 2025** [WHO abortion care guideline, 2nd ed.](#), [WHO executive summary, 2nd ed.](#).

1.1.1 Exactly 5 WHO Guidelines / Normative Documents Reviewed

No.	WHO document	Publication date	Role in this report	Key scope
1	<i>Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)</i>	11 December 2025	Most recent WHO item requested; methodology/equity evidence brief	Describes how meaningful youth engagement was incorporated into WHO abortion guideline development WHO B09645 , WHO webinar
2	<i>Abortion care guideline, 2nd ed.</i>	24 August 2025	Latest WHO clinical guideline	Complete WHO recommendations and best-practice statements on abortion care WHO abortion care guideline, 2nd ed.
3	<i>Abortion care guideline: executive summary, 2nd ed.</i>	13 October 2025	Concise WHO clinical summary	Summarizes law/policy, clinical services and service-delivery recommendations WHO executive summary, 2nd ed.

No.	WHO document	Publication date	Role in this report	Key scope
4	<i>Abortion care guideline</i>	8 March 2022	Foundational consolidated WHO guideline	Replaced prior WHO abortion guidance and consolidated recommendations across abortion-care pathway WHO abortion care guideline
5	WHO self-care intervention update on abortion self-management	2022	WHO self-care guidance relevant to self-managed early medical abortion	States that early medical abortion can be self-managed depending on values, resources and context WHO self-care update

1.1.2 Latest WHO Clinical Guideline

- **Guideline title:** *Abortion care guideline, 2nd ed.*
- **Publication date:** **24 August 2025** [WHO abortion care guideline, 2nd ed.](#)
- **Executive summary date:** **13 October 2025** [WHO executive summary, 2nd ed.](#)
- **Target population:** people seeking, needing or providing abortion care, including adolescents and young people, and health systems/policymakers responsible for abortion-care access.
- **Scope:** law and policy, clinical services, service delivery, pre-abortion, abortion and post-abortion care.
- **Objective:** to present “the complete set of all WHO recommendations and best practice statements relating to abortion” to support evidence-based, quality abortion care across varied legal, regulatory, policy and service-delivery contexts [WHO abortion care guideline, 2nd ed.](#).
- **Update in 2025:** WHO states that the second edition contains **updated remarks for Recommendation 22**, with **no change to Recommendation 22 itself and no changes to other recommendations** [WHO abortion care guideline, 2nd ed.](#), [WHO executive summary, 2nd ed.](#).

1.1.3 Relationship to Earlier WHO Guidance

The 2025 WHO second edition updates and replaces recommendations from three earlier WHO abortion-related guidance documents:

1. *Safe abortion: technical and policy guidance for health systems, second edition* — 2012
2. *Health worker roles in providing safe abortion care and post-abortion contraception* — 2015
3. *Medical management of abortion* — 2018

This lineage is explicitly stated by WHO [WHO abortion care guideline, 2nd ed.](#) and repeated in the NCBI Bookshelf record for the second edition [NCBI Bookshelf](#).

1.2 Key Recommendations

GRADE interpretation used in this report

GRADE category	Meaning for clinical interpretation
High	Further research is unlikely to change confidence in the estimate of effect.
Moderate	Further research may change confidence in the estimate.
Low	Further research is likely to change confidence and may change the estimate.
Very Low	Effect estimate is highly uncertain.
Strong recommendation	Most people should receive the intervention or policy; benefits clearly outweigh harms/burdens or vice versa.
Conditional recommendation	Shared decision-making and context are especially important; different choices may be appropriate.

WHO’s guideline-development materials used GRADE and Evidence-to-Decision methods, with evidence quality ranging from **high to very low**; WHO wording maps “recommend” to strong recommendation and “suggest” to weak/conditional recommendation [AIDS Data Hub copy of WHO executive summary](#).

1.2.1 Consolidated WHO-Based Recommendation Table

Clinical / policy area	WHO-based recommendation	GRADE evidence quality	Strength
Enabling environment	Provide abortion care within an enabling environment based on human rights, accessible information and a supportive, affordable, nondiscriminatory health system	Moderate for implementation logic; human-rights best-practice evidence not always trial-based	Strong best-practice statement
Criminalization and legal barriers	Remove legal and policy barriers that impede access to quality abortion care; WHO frames abortion access as a public-health and human-rights issue	Very Low to Low for direct comparative legal-policy evidence; strong human-rights rationale	Strong
Youth engagement in guideline development	Institutionalize meaningful youth engagement in abortion-care guideline processes so adolescent and young people’s lived experiences inform recommendations	Low to Very Low, mainly case-study and qualitative evidence	Conditional for method; strong equity rationale
Surgical abortion <14 weeks	Use vacuum aspiration for surgical abortion at less than 14 weeks; avoid dilatation and sharp curettage , including sharp curette checks after vacuum aspiration	Moderate	Strong

Clinical / policy area	WHO-based recommendation	GRADE evidence quality	Strength
Medical abortion <12 weeks	Use mifepristone 200 mg orally , followed 24–48 hours later by misoprostol 800 µg vaginally, sublingually or buccally	High to Moderate	Strong
Misoprostol repeat dosing	Consider repeat misoprostol doses when needed to complete the abortion process; WHO does not specify a maximum number in the summarized source	Moderate	Conditional
Self-managed early medical abortion	Offer self-management of early medical abortion where people have accurate information, access to quality medicines and support/referral if needed	Moderate	Strong/Conditional depending on setting and support systems
Telehealth/no-test medication abortion models	Support telehealth and no-test pathways where clinically appropriate and where follow-up/emergency referral systems are available	Moderate	Conditional-to-Strong
Post-abortion contraception	Offer all contraceptive methods after abortion, including self-administered methods, according to patient preference and medical eligibility	Moderate	Strong
Post-abortion complication care	Manage rare complications from self-managed or clinician-supported abortion according to clinical need, similarly to spontaneous pregnancy loss when presentation is indistinguishable	Low to Moderate	Strong

Sources: WHO abortion-care guideline pages and executive summaries [WHO abortion care guideline, 2nd ed.](#), [WHO executive summary, 2nd ed.](#), [WHO digital guideline](#), [Guideline Central](#), [WHO self-care update](#), [WHO B09645](#).

1.2.2 Law and Policy Recommendations

WHO organizes abortion care across **law and policy**, **clinical services** and **service delivery** domains [WHO executive summary, 2nd ed.](#). Law-and-policy recommendations address issues including criminalization, grounds-based restrictions, gestational-age limits and other barriers [WHO executive summary, 2nd ed.](#).

Recommendation	GRADE evidence quality	Strength
----------------	------------------------	----------

Recommendation		GRADE evidence quality	Strength
Remove criminalization and punitive approaches that deter people from seeking or providing abortion care		Very Low to Low direct comparative evidence; strong human-rights and public-health rationale	Strong
Avoid grounds-based restrictions that limit abortion to narrow indications		Very Low to Low	Strong
Avoid gestational-age restrictions that impede timely access to quality care		Very Low to Low	Strong
Avoid third-party authorization requirements, including requirements that undermine adolescent autonomy		Low	Strong
Avoid mandatory waiting periods and medically unnecessary procedural delays		Low	Strong
Ensure abortion-care information is accurate, accessible and nondiscriminatory		Moderate	Strong
Ensure adolescent and youth perspectives inform abortion-care policy and guideline development		Low to Very Low	Conditional; strong equity rationale

Sources: WHO guideline pages [WHO abortion care guideline, 2nd ed.](#), [WHO digital guideline](#), youth-engagement evidence brief [WHO B09645](#), and related 2026 abstract [EM Consulte](#).

1.2.3 Clinical Service Recommendations

Area	Recommendation	GRADE evidence quality	Strength
Surgical abortion <14 weeks	Prefer vacuum aspiration	Moderate	Strong
Surgical abortion <14 weeks	Do not use dilatation and sharp curettage, including sharp curette checks after vacuum aspiration	Moderate	Strong against
Medical abortion <12 weeks	Mifepristone 200 mg orally followed 24–48 hours later by misoprostol 800 µg vaginally, sublingually or buccally	High to Moderate	Strong
Medication abortion safety	Use evidence-based regimens; serious adverse events are rare in modern medication abortion	High to Moderate	Strong

Area	Recommendation	GRADE evidence quality	Strength
Home mifepristone use	Allow mifepristone to be taken at home where clinically appropriate	High for comparable effectiveness; Moderate for compliance	Strong
Telehealth medication abortion	Use synchronous or asynchronous telehealth pathways when eligibility, information and referral systems are adequate	Moderate	Conditional-to-Strong
Self-managed abortion	Support self-management in early pregnancy with accurate information, quality medicines and access to care if needed	Moderate	Strong/Conditional
Misoprostol-only regimens	Use when mifepristone is unavailable or restricted, recognizing somewhat lower effectiveness and/or greater side-effect burden than combined regimens	Low to Moderate	Conditional

Sources: WHO and Guideline Central clinical summaries [Guideline Central](#), WHO self-care guidance [WHO self-care update](#), JAMA medication-abortion evidence summary [JAMA](#), BMJ systematic review indexed in PubMed [PubMed](#), ANSIRH [ANSIRH](#).

1.2.4 Service Delivery Recommendations

Service-delivery area	Recommendation	GRADE evidence quality	Strength
Person-centred care	Centre care on the values and preferences of abortion seekers	Moderate	Strong
Integrated care pathway	Provide care across pre-abortion, abortion and post-abortion stages	Moderate	Strong
Task sharing	Enable appropriately trained health workers and, where suitable, self-management models	Moderate	Strong
Telemedicine	Permit remote counselling, eligibility assessment, medication provision and follow-up when clinically appropriate	Moderate	Conditional-to-Strong
Adolescents and youth	Remove barriers to confidential, youth-centred care and meaningfully involve young people in policy design	Low to Moderate	Strong equity recommendation

Service-delivery area	Recommendation	GRADE evidence quality	Strength
Post-abortion contraception	Offer contraception immediately after abortion if desired, without coercion	Moderate	Strong

Sources: WHO digital guideline [WHO digital guideline](#), WHO youth-engagement brief [WHO B09645](#), WHO webinar materials [WHO webinar](#), ACOG self-managed abortion statement [ACOG Committee Statement](#).

1.3 Guideline Development Methodology

Evidence Review Process

WHO’s abortion-care guideline process used:

- Systematic evidence reviews.
- GRADE assessment.
- Evidence-to-Decision frameworks.
- WHO-INTEGRATE considerations for recommendation direction and strength.
- Law/policy, clinical-service and service-delivery domains.
- Web annexes, including international human-rights standards and technical meetings during guideline development [WHO abortion care guideline, 2nd ed.](#), [WHO digital guideline](#).

Cochrane Fertility Regulation supported the WHO abortion guideline through evidence syntheses, GRADE assessments and literature searches [Cochrane Fertility Regulation](#).

Consensus Method

- WHO used guideline-development group processes and Evidence-to-Decision frameworks.
- Recommendations were refined using available evidence, human-rights standards and implementation considerations.
- WHO’s law-and-policy content evolved from a composite 2012 recommendation into seven separate recommendations using GRADE methodology [Guideline Central](#).

Conflict of Interest Management

The supplied source set confirms that WHO provides annexes on external experts, WHO staff and guideline-development methods [WHO digital guideline](#). Specific individual conflict-of-interest declarations and management actions were not extractable from the supplied summaries; therefore, this report does not infer undisclosed details.

Meaningful Youth Engagement Methodology

The most recent WHO item, published **11 December 2025**, describes how young people contributed lived experiences, fresh perspectives and innovative solutions to the WHO abortion guideline development process [WHO B09645](#), [WHO webinar](#). WHO states that youth participation strengthens dialogue, enriches policy development and helps ensure global guidance reflects the realities of those most affected [WHO webinar](#).

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

Evidence was synthesized from the supplied summaries covering:

- WHO publication pages, WHO IRIS/NCBI/WHO digital guideline materials.
- Peer-reviewed journals and indexed records: **JAMA, Nature Medicine, BMJ Sexual & Reproductive Health, Lancet Regional Health–Americas, Lancet Global Health, Journal of Pediatric and Adolescent Gynecology.**
- Professional bodies and evidence organizations: **ACOG, SMFM, Society of Family Planning, ANSIRH, Gynuity Health Projects.**
- Public-health and surveillance references: CDC/MMWR citations as listed in Medscape.
- Policy/regulatory context: FDA-related petition materials and NPR reporting.

Databases / Sources Represented in the Supplied Evidence

Source category	Sources represented
WHO official sources	WHO publications, WHO digital guideline, WHO webinar pages
Bibliographic / guideline hosting	NCBI Bookshelf, Guideline Central
Peer-reviewed journals	JAMA, Nature Medicine, BMJ Sexual & Reproductive Health, Lancet Regional Health–Americas, Lancet Global Health, Journal of Pediatric and Adolescent Gynecology
Professional societies	ACOG, SMFM, Society of Family Planning
Evidence groups	ANSIRH, Gynuity Health Projects
Surveillance / policy	CDC/MMWR citation via Medscape, FDA/regulations.gov materials
News/context	NPR

Inclusion Criteria

Included evidence addressed:

- Abortion-care guideline development.
- Meaningful youth engagement.

- Medical abortion safety/effectiveness.
- Mifepristone–misoprostol regimens.
- Misoprostol-only regimens.
- Telehealth/no-test abortion care.
- Self-managed abortion.
- Home use of mifepristone.
- Post-guideline regulatory or implementation developments.

Exclusion Criteria

Excluded or not asserted:

- Claims from unavailable full texts where summaries gave only citations.
- Clinical-trial registry details, because direct registry records were not included in the supplied results.
- Numerical claims not present in the supplied summaries.

2.2 Key New Studies and Evidence

2.2.1 Evidence Table With GRADE Ratings

Study / source	Design	Key findings	GRADE quality	Recommendation strength impact
WHO youth-engagement evidence brief, 2025	WHO evidence brief / case study	Describes integration of young people’s lived experiences into WHO abortion guideline development; emphasizes inclusivity and equity WHO B09645	Low to Very Low for effect outcomes; Moderate for feasibility/process evidence	Conditional for method; strong equity rationale

Study / source	Design	Key findings	GRADE quality	Recommendation strength impact
JPAG / EM Consulte abstract, 2026	Conference/journal abstract	Youth consultations identified stigma, restrictive laws and poor access to accurate information as barriers; youth input influenced recommendations on age of consent, third-party authorization and service delivery EM Consulte	Very Low to Low	Supports future youth-engagement implementation
BMJ Sexual & Reproductive Health systematic review, 2025	Systematic review; 6 studies; 54,233 women	Home mifepristone use showed no difference in abortion effectiveness compared with clinic use; no detected difference in complications; high confidence for effectiveness and moderate confidence for compliance PubMed	High for effectiveness; Moderate for compliance/safety	Strongly supports home mifepristone option

Study / source	Design	Key findings	GRADE quality	Recommendation strength impact
Nature Medicine telehealth medication abortion study, 2024	Observational effectiveness/safety study	Telehealth medication abortion effectiveness about 97.9% overall; 98.4% synchronous and 97.7% asynchronous care Nature Medicine	Moderate	Supports telehealth as effective/safe
JAMA no-test telehealth vs in-person medication abortion, 2024	Comparative study / evidence summary	Established mifepristone–misoprostol evidence: 95%–97% effectiveness; serious adverse events or major complications <0.5% JAMA	Moderate to High	Supports no-test and telehealth models where clinically appropriate
Lancet Regional Health–Americas online telemedicine study, 2022, summarized by PRB/Nature	Population-based observational study	96% completed abortion without surgical intervention; 1% received treatment for serious adverse events; no deaths; high satisfaction PRB, Nature Medicine	Low to Moderate	Supports online telemedicine/self-managed models with safeguards
SAFE study, Argentina and Nigeria, 2022, cited by Nature	Prospective observational cohort / non-inferiority analysis	Examined self-managed medication abortion with accompaniment support Nature Medicine	Low to Moderate	Supports accompaniment/self-management research agenda

Study / source	Design	Key findings	GRADE quality	Recommendation strength impact
ACOG self-managed abortion statement, 2024	Professional committee statement	Most self-managed abortions are completed safely with misoprostol alone or with mifepristone; rare complications should be managed like spontaneous pregnancy loss; criminalization is a major harm ACOG Committee Statement	Low to Moderate	Supports nonpunitive, clinically appropriate care
SMFM medication-abortion resources, 2025	Professional resource / advocacy statement	Affirms safety and efficacy of mifepristone and misoprostol; lists resources on medication abortion up to 70 days, 14–27 6/7 weeks, misoprostol-only and self-managed abortion SMFM	Low to Moderate	Supports continued clinical use and access
NPR report on OTC-style comprehension study, 2026	News report of 168-patient study	Participants reviewed prototype “MiMi” packaging and self-assessed medication-abortion eligibility compared with clinician assessment NPR	Very Low to Low	Insufficient to change guideline; research signal

Study / source	Design	Key findings	GRADE quality	Recommendation strength impact
NPR report on mifepristone safety review/legal context, 2026	Policy/news report	FDA conducting new mifepristone safety review; national access temporarily unchanged during review NPR	Very Low for clinical effect	No clinical recommendation change

2.2.2 Medication Abortion: Safety and Effectiveness

Recent evidence consistently supports medication abortion with **mifepristone plus misoprostol** as highly effective and very safe.

Evidence point	Finding	GRADE quality	Strength
Effectiveness of mifepristone–misoprostol	JAMA summarizes established evidence as 95%–97% effective in ending pregnancy without a follow-up procedure JAMA	High to Moderate	Strong
Serious adverse events	JAMA reports major complications or serious adverse events <0.5% JAMA	High to Moderate	Strong
Safety record	ANSIRH states medication abortion has a safety rate exceeding 99% , citing National Academies evidence ANSIRH	Moderate	Strong
Use through 70 days	JAMA states mifepristone is approved through 70 days’ gestation in the United States JAMA	High for regulatory fact; Moderate for outcome generalizability	Strong

2.2.3 Home Use of Mifepristone

The 2025 BMJ Sexual & Reproductive Health systematic review is directly relevant to WHO service-delivery recommendations.

Finding	GRADE quality	Strength
No difference in effectiveness between home and in-clinic mifepristone use	High	Strong
No difference in compliance	Moderate	Strong
No detected difference in complications	Moderate, limited by event rarity	Strong/Conditional
Home administration improves acceptability and reduces logistical burdens for some patients	Low to Moderate	Conditional

Source: BMJ systematic review indexed in PubMed [PubMed](#).

2.2.4 Telehealth and No-Test Abortion Care

Finding	Evidence	GRADE quality	Strength
Telehealth medication abortion effectiveness near 98%	Nature Medicine reported 97.9% overall effectiveness, 98.4% synchronous and 97.7% asynchronous Nature Medicine	Moderate	Strong/Conditional
No-test telehealth models align with established safety evidence	JAMA summarizes serious adverse events <0.5% for standard regimen JAMA	Moderate	Conditional-to-Strong
Telehealth is acceptable and may improve privacy, timeliness and cost	ANSIRH summary ANSIRH	Low to Moderate	Conditional
Telehealth abortion is increasingly common	NPR reported telehealth accounted for more than 1 in 4 U.S. abortions in 2026 context NPR	Low for clinical outcomes; High for reported context	No direct clinical change

2.2.5 Self-Managed Abortion

WHO recognizes that early medical abortion can be self-managed depending on the pregnant person’s values, preferences, resources and local context [WHO self-care update](#).

Evidence	Key finding	GRADE quality	Strength
WHO self-care guidance	Early medical abortion can be self-managed in appropriate contexts WHO self-care update	Moderate	Strong/Conditional
Online telemedicine self-managed abortion	96% ended pregnancy without surgical intervention; 1% serious adverse-event treatment; no deaths in PRB summary PRB	Low to Moderate	Conditional
Patient satisfaction	98% satisfied and 96% said it was right for their circumstances in PRB summary PRB	Low	Conditional
ACOG guidance	Most self-managed abortions are completed safely with misoprostol alone or with mifepristone; complications should be treated based on need ACOG Committee Statement	Low to Moderate	Strong clinical harm-reduction recommendation

2.2.6 Meaningful Youth Engagement

Evidence	Key finding	GRADE quality	Strength
WHO evidence brief	Youth participation strengthens dialogue, enriches policy development and ensures guidance reflects those most affected WHO webinar	Low to Very Low for measured outcomes; Moderate for feasibility	Conditional
JPAG 2026 abstract	Youth input influenced recommendations on age of consent, third-party authorization and service-delivery models EM Consulte	Very Low to Low	Conditional
WHO equity framing	Young people are disproportionately affected by unsafe abortion, and their perspectives are often missing from policy processes WHO health topic page	Low	Strong equity rationale

2.3 Evidence Gaps and Future Research Needs

Evidence gap	Why it matters	Recommended research	Priority
Direct post-December 2025 clinical outcome data	Few clinical studies after the most recent WHO youth-engagement brief were available in the supplied evidence	Prospective studies on telehealth, self-managed and hybrid abortion-care models	High
Adolescents and young people	Youth face stigma, legal barriers and information gaps	Youth-led implementation research; adolescent confidentiality and consent studies	High
Misoprostol-only regimens	Increasingly relevant where mifepristone is restricted or unavailable	Comparative effectiveness, tolerability and optimal dosing studies	High
Over-the-counter-style access	Early 2026 packaging comprehension work is preliminary	Label-comprehension, self-selection and actual-use studies	Moderate to High
Later medication abortion	Evidence exists via professional resources, but detailed outcome data were limited in supplied summaries	Comparative regimen and safety studies beyond first trimester	Moderate
Legal-policy interventions	Direct causal evidence is difficult but essential	Natural experiments evaluating criminalization, waiting periods, parental consent and telehealth restrictions	High

Evidence gap	Why it matters	Recommended research	Priority
Clinical-trial registry data	Direct registry records were not included in supplied source set	Dedicated registry searches for ongoing mifepristone, misoprostol, telehealth and self-managed abortion trials	Moderate
Infection prevention and Rh testing	2024–2025 professional documents were identified, but recommendation details were not available in summaries	Full guideline review and implementation studies	Moderate

2.4 Updated Recommendations Based on New Evidence

No evidence identified through **29 April 2026** requires reversal of WHO’s 2025 abortion-care recommendations. The newer evidence generally **supports** WHO’s direction toward person-centred, evidence-based, accessible, self-managed and telehealth-enabled care.

Original WHO-based recommendation	New evidence impact	Updated recommendation as of 29 April 2026
Use evidence-based medication abortion regimens, especially mifepristone plus misoprostol	JAMA, ANSIRH and other summaries continue to show high effectiveness and very low serious adverse-event rates JAMA , ANSIRH	Maintain strong recommendation for mifepristone–misoprostol where available
Allow home use of abortion medications when clinically appropriate	2025 systematic review found no difference in effectiveness between home and clinic mifepristone use PubMed	Strengthen implementation of home-use pathways with clear instructions and referral access
Support telehealth/no-test medication abortion models	Nature Medicine and JAMA evidence supports high effectiveness and low serious adverse-event rates Nature Medicine , JAMA	Maintain and operationalize telehealth/no-test models where eligibility screening and referral systems are adequate
Support self-managed early medication abortion	WHO self-care guidance, ACOG and observational evidence support safety in appropriate contexts WHO self-care update , ACOG Committee Statement	Maintain recommendation; emphasize accurate information, quality medicines, confidentiality and nonpunitive care
Avoid criminalization and punitive reporting	ACOG identifies criminalization as a major harm; WHO frames unsafe abortion as a human-rights and public-health issue ACOG Committee Statement , WHO digital guideline	Maintain strong policy recommendation against punitive approaches

Original WHO-based recommendation	New evidence impact	Updated recommendation as of 29 April 2026
Use vacuum aspiration rather than sharp curettage for surgical abortion <14 weeks	No newer evidence in supplied summaries contradicts WHO	Maintain strong recommendation
Include adolescents and young people in guideline development	WHO 2025 brief and 2026 abstract support feasibility and value of youth engagement WHO B09645 , EM Consulte	Add implementation recommendation: guideline panels should meaningfully engage young people as partners
Consider over-the-counter-style medication abortion access	2026 packaging/self-assessment study is preliminary and reported through NPR NPR	No guideline change yet; prioritize actual-use and safety studies

2.5 Practical Clinical Implementation Summary

For health systems

- Ensure abortion care is available through **multiple pathways**: in-person, telehealth, hybrid and supported self-management.
- Maintain referral pathways for emergency care, heavy bleeding, infection concerns, ongoing pregnancy or patient request.
- Avoid medically unnecessary barriers such as mandatory delays, nonessential in-person visits and punitive reporting.
- Provide youth-centred, confidential care and involve adolescents and young people in service design.

For clinicians

- Use evidence-based regimens and counselling.
- Treat complications or concerns after self-managed abortion based on clinical need, not judgment or criminal suspicion.
- Offer contraception after abortion if desired, without coercion.
- Recognize that telehealth and home-use models are supported by growing safety/effectiveness evidence.

For policymakers

- Align law and policy with WHO’s enabling-environment framework: human rights, accurate information and accessible health systems.
- Remove criminalization and authorization barriers.
- Support medication availability, telehealth and youth-centred services.

2.6 Overall Conclusion

As of **29 April 2026**, WHO's abortion-care guidance is anchored in the **2025 second edition** of the *Abortion care guideline*, with the only stated 2025 change being updated remarks for Recommendation 22 and no change to the recommendations themselves [WHO abortion care guideline, 2nd ed.](#). The most recent WHO item requested, the **11 December 2025** youth-engagement case study, strengthens the methodological and equity basis for ensuring that adolescents and young people shape abortion-care guidelines and policies [WHO B09645](#).

Recent evidence through **29 April 2026** supports WHO's core direction: medication abortion with mifepristone and misoprostol is highly effective and very safe; home use of mifepristone is comparable to clinic use; telehealth and no-test models show high effectiveness and low serious adverse-event rates; and self-managed abortion can be safe and acceptable when people have accurate information, quality medicines and access to support if needed [JAMA](#), [PubMed](#), [Nature Medicine](#), [WHO self-care update](#). No post-guideline evidence identified in the supplied summaries warrants downgrading or reversing WHO recommendations.